

JCOIN 2.0 Proposed Baseline Staff and Organizations Core Measures Specifications for PI Input

Goal: Identify a set of “Core” staff and organization level CDE recommended for all JCOIN 2 studies that are prospectively collecting patient level data in order to build a “pooled” data set for secondary analysis.

TOTAL NUMBER OF CDE ITEMS

DOMAIN	CORE ITEMS	OPTIONAL ITEMS
1. Staff Demographics	9	0
2. Brief Opioid Stigma Survey (BOSS)	0	12
3. Lived and Living Experience	0	13
4. MOUD Knowledge/Perceptions (Staff measure)	0	28
5. Occupational Stress/Burnout (Staff)	0	4
6. Organizational Climate: Change Readiness/Resistance	0	16
7. Implementation Outcomes (Organizational Measure)	0	12
8. BJA Cascade of Care Questions	0	12
9. Organizational Level Metadata	20	0
Total	29	97

Below please find the proposed specifications for the JCOIN 2.0 Baseline Client Core Data Elements. This document is organized by domain headers. Each row is followed by the proposed core data elements as survey items, and, if applicable, optional items to be integrated **highlighted in yellow**. The required number of items per subdomain are in **bold** and optional items in parentheses ().

In all questions, the following codes for missing values apply:

Codes for Missing Data	
Code / Value	Label
.a	Don't know
.b	Refused
.c	Left blank
.d	Legitimately skipped
.e	Not collected
.f	Missing in error
.g	Redacted

1 STAFF DEMOGRAPHIC CHARACTERISTICS

<i>Demographics</i>	Age	Age in years	1	
D1 What is your current age in years?			___ ___ Years	
<i>Demographics</i>	Sex at Birth	Sex at Birth	1	
D2. What sex were you assigned at birth, on your original birth certificate?			1. Female 2. Male 3. Intersex 4. None of these describe me	
<i>Demographics</i>	Ethnicity/Race		1	
D3. What is your race and/or ethnicity? (Select all that apply)			1. American Indian or Alaska Native 2. Asian, Asian American, or Southeast Asian, including Chinese, Japanese, and others 3. Black or African American 4. Hispanic or Latino(a) or Latine/x 5. Middle Eastern or North African 6. Native Hawaiian or Other Pacific Islander 7. White or Caucasian 8. Something else: _____	
<i>Demographics</i>	Education	Years of highest education	1	
D4. What is the highest grade or level of school you have completed or the highest degree you have received?			1. Did not complete high school (enter highest grade level completed: _____) 2. High school diploma, GED, or equivalent 3. Some technical or college, but no certificate or degree 4. Technical/trade/certificate school or program 5. Some Associate or Bachelor's work, but no degree 6. Associate's Degree (e.g., AA or AS) 7. Bachelor's Degree (e.g., BA or BS) 8. Some graduate work, but no degree 9. Graduate Degree (e.g., Masters, Doctorate, other)	
<i>Demographics</i>	Employment	Primary Workplace	4	
D5. What is your primary workplace?			☐ Criminal justice, legal system, or public safety setting ☐ Health care, behavioral health, or social services setting	
D5a. If a criminal justice, legal system, or public safety setting Do you work in a healthcare, behavioral health, or social services role?			☐ Yes ☐ No	

<i>D5a1. If the setting or role is Health Care, Behavioral Health, or Social Service Delivery options</i> Which of the following best describes your primary professional role in your current job? (select one)	☐ Physician, Nurse Practitioner, Physician Assistant, or other Prescribing Medical Professional ☐ Pharmacist/pharmacy technician ☐ Registered Nurse or Licensed Practical Nurse ☐ Certified or licensed mental or behavioral health professional (e.g., LCPC, LPC, LCSW, LSW LMFT, LPC, Addictions Counselor). ☐ Peer Recovery Support Specialist / Recovery Support Staff ☐ Non-certified or non-licensed mental or behavioral health professional ☐ Case Manager ☐ Crisis Response / first responder ☐ Support and coordination Staff (e.g., intake, care coordination outreach, linkage manager) ☐ Other role not listed → specify
<i>D5a2. If the setting is Criminal Justice, Legal System, or Public Safety and the role is not Health Care, Behavioral Health, or Social Service Delivery options</i> Which of the following best describes your primary professional role in your current job? (select one)	☐ Judge or Judicial Officer ☐ Legal counsel (e.g., prosecutor, public defender) ☐ Court Leadership and Staff (e.g., coordinator/administrator, security, bailiff, court reporter, pre-trial officer) ☐ Court offered peer services or service navigation ☐ Law Enforcement Leadership and Staff (e.g., chief, sheriff officer) ☐ Probation, Parole, or Community Supervision Leadership and Staff (e.g., supervisor, officer) ☐ Prison Leadership and Staff (e.g., warden, prison officer/corrections officer, security) ☐ Jail Leadership and Staff (e.g., warden, correctional officer/detention officer, security) ☐ Other role not listed → specify
D6. Do you hold a professionally recognized license or certification directly related to your job?	☐ Yes ☐ No
D6a. If Yes, which? (check all that apply)	☐ Medical license/credential (physician, nurse, EMT, prescriber) ☐ Counseling credential (e.g., LCSW, addiction counselor) ☐ Peer certification / Recovery support specialist (or similar) ☐ Corrections certification ☐ Other (specify):

OPTIONAL QUESTIONS

2 BRIEF OPIOID STIGMA SCALE (BOSS)

BRIEF OPIOID STIGMA SCALE (BOSS) — STEREOTYPE AGREEMENT SUBSCALE (STAFF MEASURE)¹ OPTIONAL	
Note: We recommend hubs use the first “opioid” item. It is the original item, which has strong scientific evidence supporting its use. The other two items are based on suggestions from the workgroup listed below, and we recommend they only be used in addition to the “opioid” item.)	
B1. I believe that a person who uses opioids (including prescription painkillers like oxycodone, hydrocodone, morphine, codeine, fentanyl, and tramadol, as well as the illegal drug heroin)	
B1a.cannot be trusted	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B1b.is to blame for their problems	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B1c. is lazy	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B1d. Is dangerous	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree

¹ Yang, L. H., Grivel, M. M., Anderson, B., Bailey, G. L., Opler, M., Wong, L. Y., & Stein, M. D. (2019). A new brief opioid stigma scale to assess perceived public attitudes and internalized stigma: Evidence for construct validity. *Journal of Substance Abuse Treatment*, 99, 44–51. <https://doi.org/10.1016/j.jsat.2019.01.005>; The stereotype agreement subscale of the Brief Opioid Stigma Scale (BOSS) measures individuals’ endorsement of stigmatizing beliefs about people with opioid use disorder and is one of three four-item subscales within the 12-item instrument, alongside stereotype awareness and self-devaluation. Although initially validated among U.S. adults with opioid use disorder, it is widely used to assess stigmatizing attitudes in broader populations, including healthcare, social service, and justice-system staff, capturing key dimensions of opioid-related stigma such as moral blame, negative character judgments, and perceived dangerousness. The subscale demonstrates acceptable psychometric properties as a distinct factor within a three-factor model, with internal consistency estimates of Cronbach’s $\alpha \approx 0.68$ – 0.72 and evidence of construct validity through associations with depression and self-esteem, and is freely available for use in U.S. samples.

B2. I believe that a person who uses illegal/illicit drugs...	
B2a.cannot be trusted	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B2b.is to blame for their problems	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B2c. is lazy	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B2d. Is dangerous	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B3. I believe that a person who uses stimulants such as methamphetamine or cocaine/crack...	
B3a.cannot be trusted	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B3b.is to blame for their problems	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B3c. is lazy	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree
B3d. Is dangerous	1 = Strongly disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Strongly agree

3 LIVED & LIVING EXPERIENCE

LIVED AND LIVING EXPERIENCE: FAMILY & FRIENDS (STAFF MEASURE)	
OPTIONAL	
<i>There are two response scale options, depending on what the hub prefers. Note that items L1-L7 will all use the same response options chosen by the hub (i.e., either Option 1 for all 7 items or Option 2 for all 7 items)</i> <u>Option 1:</u> Select a response for each experience that applies, even if it is for the same person. As a reminder, your answers are confidential. ☐ yes, ☐ no, ☐ I'm unsure <u>Option 2:</u> Check all that apply by selecting the relevant relationship(s). As a reminder, your answers are confidential. ☐ No close family or friends with these experiences; ☐ A close family member ☐ A significant other, ☐ A close friend, ☐ I'm unsure	
Please indicate whether you have any CLOSE family members, significant others, or CLOSE friends who have had the following experiences. By CLOSE, we mean someone you have regular contact with or feel emotionally connected to.	
L1. Been diagnosed with an alcohol or drug use disorder.	
L2. Participated in addiction treatment or recovery support services.	
L3. Currently in recovery from an alcohol or drug use disorder.	
L4. Survived an overdose.	
L5. Death from an overdose.	
L6. Criminal-legal system involvement (e.g., arrest, probation, incarceration).	
L7. Diagnosed mental health condition such as major depression, bipolar disorder, psychotic disorder, schizophrenia, or schizoaffective disorder.	
LIVED AND LIVING EXPERIENCE: SELF	
OPTIONAL	
Please indicate whether you have personally experienced any of the following at any point in your life. As a reminder, your answers are confidential (check all that apply)	
L8. I have been diagnosed with an alcohol or drug use disorder.	☐ Yes ☐ No ☐ I'm unsure
L9. I have participated in addiction treatment or recovery support services.	☐ Yes ☐ No ☐ I'm unsure
L10. I am currently in recovery from a drug or alcohol use disorder.	☐ Yes ☐ No ☐ I'm unsure
L11. I have survived an overdose.	☐ Yes ☐ No ☐ I'm unsure
L12. I have been involved in the criminal-legal system (e.g., been arrested, on probation, and/or incarcerated).	☐ Yes ☐ No ☐ I'm unsure
L13. I have been diagnosed with a mental health condition, such as major depression, bipolar disorder, psychotic disorder, schizophrenia, or schizoaffective disorder.	☐ Yes ☐ No ☐ I'm unsure

4 MOUD KNOWLEDGE/PERCEPTIONS (STAFF MEASURE)²

OPINIONS ABOUT MEDICATION FOR ADDICTION TREATMENT (OAMAT)	
OPTIONAL	
Because treatments can be used for different indications, there was strong sentiment that the specific substance use disorder should be defined in the context of each treatment. Note: For hubs that plan to ask about MOUD and any other treatments using the same item set, the items would need to specify the condition being treated. This structure would also allow hubs to add treatments for alcohol, pain, withdrawal, diabetes/weight (GLP1s), etc. if that's relevant to their study. The items would follow the same structure as outlined for items M1a-M5d above, substituting the condition of interest for "opioid use disorder."	
Please rate your level of agreement with the following statements.	
M1. I am <u>familiar</u> with the following treatments for opioid use disorder...	
M1a. Methadone	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M1b. Buprenorphine – tablets or film dissolved by mouth (Suboxone, Zubsolv)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M1c. Buprenorphine – long-acting injectable (weekly or monthly) (Sublocade, Brixadi)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M1d. Naltrexone – long-acting injectable (monthly) (Vivitrol)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M2. I have received training about the following treatments for opioid use disorder	
M2a. Methadone	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree

² This is an updated version of: Friedmann et al., 2015. Effect of an organizational linkage intervention on staff perceptions of medication-assisted treatment and referral intentions in community corrections. JSAT 50:50-58.

M2b. Buprenorphine – tablets or film dissolved by mouth (Suboxone, Zubsolv)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M2c. Buprenorphine – long-acting injectable (weekly or monthly) (Sublocade, Brixadi)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M2d. Naltrexone – long-acting injectable (monthly) (Vivitrol)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M3. I think this treatment is helpful to individuals with opioid use disorder:	
M3a. Methadone	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M3b. Buprenorphine – tablets or film dissolved by mouth (Suboxone, Zubsolv)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M3c. Buprenorphine – long-acting injectable (weekly or monthly) (Sublocade, Brixadi)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M3d. Naltrexone – long-acting injectable (monthly) (Vivitrol)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M4. I know where to refer individuals to get this treatment for opioid use disorder: (Hubs may omit this item if referrals are not relevant to the study setting/aims.)	
M4a. Methadone	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree

M4b. Buprenorphine – tablets or film dissolved by mouth (Suboxone, Zubsolv)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M4c. Buprenorphine – long-acting injectable (weekly or monthly) (Sublocade, Brixadi)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M4d. Naltrexone – long-acting injectable (monthly) (Vivitrol)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M5. I am willing to refer individuals to programs/providers offering this treatment for opioid use disorder: (Hubs may omit this Item if referrals are not relevant to the study setting/aims.)	
M5a. Methadone	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M5b. Buprenorphine – tablets or film dissolved by mouth (Suboxone, Zubsolv)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M5c. Buprenorphine – long-acting injectable (weekly or monthly) (Sublocade, Brixadi)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M5d. Naltrexone – long-acting injectable (monthly) (Vivitrol)	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree

PERCEPTIONS AND KNOWLEDGE ABOUT MOUD	
OPTIONAL	
Please indicate your level of agreement with each of the following statements about Medications for Opioid Use Disorder (MOUD), sometimes also known as MAT (Medications for Addiction Treatment).	
M6. MOUD reduces the effects of opioids.	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M7. People should be able to take MOUD as a lifelong treatment.* <i>*Hubs may choose to ask either M7 or M8 (or both).</i>	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M8. People should aim to taper off MOUD and continue their recovery without medication*.[R] <i>*Hubs may choose to ask either M7 or M8 (or both).</i>	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M9. MOUD is just substituting one drug for another. [R]	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M10. Ongoing use of MOUD reduces a person's criminal activities.	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M11. Ongoing use of MOUD reduces a person's risk of dying.	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M12. MOUD reduces a person's consumption of illicit opioids.	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree
M13. People do not need MOUD after jail/prison because there is no drug use in jail/prison. [R] <i>Hubs may opt out of this item depending on their study setting.</i>	1 = Strongly Disagree 2 = Disagree 3 = Neither Agree nor Disagree 4 = Agree 5 = Strongly Agree

5 OCCUPATIONAL STRESS/BURNOUT (STAFF)³

BURNOUT ASSESSMENT TOOL (BAT4)	
OPTIONAL	
S1. At work, I feel mentally exhausted.	1 = Never 2 = Rarely 3 = Sometimes 4 = Often 5 = Always
S2. I struggle to find any enthusiasm for my work.	1 = Never 2 = Rarely 3 = Sometimes 4 = Often 5 = Always
S3. When I'm working, I have trouble concentrating.	1 = Never 2 = Rarely 3 = Sometimes 4 = Often 5 = Always
S4. At work, I may overreact unintentionally.	1 = Never 2 = Rarely 3 = Sometimes 4 = Often 5 = Always

³ Hadžibajramović, E., Schaufeli, W., & De Witte, H. (2024). The ultra-short version of the Burnout Assessment Tool (BAT4)–development, validation, and measurement invariance across countries, age and gender. PLoS ONE, 19(2), e0297843; The burnout subscale of the Survey of Organizational Functioning (TCU SOF) demonstrates strong psychometric properties, with studies consistently reporting good internal consistency (typically Cronbach's α in the .75–.85 range) and a clear, coherent factor structure within the broader organizational climate domain. The scale shows robust construct validity, evidenced by expected associations with workload, job stress, staff morale, turnover intentions, and perceived organizational support, and has proven sensitive to organizational change efforts, making it well suited for use in workforce well-being, implementation, and quality-improvement research. <https://doi.org/10.1371/journal.pone.0297843>; The BAT was developed to assess burnout complaints across multiple core dimensions, rather than to serve as a diagnostic tool alone. It was built from a large item pool to capture core and secondary symptoms of burnout, as defined in theory and empirical work. There are 23, 12, and 4-item versions. It has validated English-language versions, and the strongest psychometric evidence comes from European and other international samples, with English versions primarily tested outside the U.S. The four-item Burnout Assessment Tool (BAT-4) was developed from the validated 12-item version by selecting one strong item from each core burnout area, reducing respondent burden. Studies show that the BAT-4 measures a single overall burnout level and performs similarly across countries, age groups, and genders, making it appropriate for group-level comparisons. While it cannot provide detailed subscale scores, it offers a practical alternative to the 12-item version.

6 ORGANIZATIONAL CLIMATE: CHANGE READINESS/RESISTANCE (ORGANIZATIONAL MEASURE)⁴

ORGANIZATIONAL READINESS FOR IMPLEMENTING CHANGE (ORIC; JCOIN 1.0 OPTIONAL MEASURE)	
OPTIONAL	
Instructions: Replace "[intervention]" with the name of the program or practice being implemented.	
O1. People who work here feel confident that the organization can get people invested in implementing [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O2. People who work here are committed to implementing [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O3. People who work here feel confident that they can keep track of progress in implementing [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O4. People who work here will do whatever it takes to implement [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O5. People who work here feel confident that the organization can support people as they adjust to [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O6. People who work here want to implement [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O7. People who work here feel confident that they can keep the momentum going in implementing [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O8. People who work here feel confident that they can handle the challenges that might arise in implementing [intervention] .	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree

⁴ Lehman, W. E. K., Greener, J. M., & Simpson, D. D. (2002). Assessing organizational readiness for change. *Journal of Substance Abuse Treatment*, 22(4), 197–209. [https://doi.org/10.1016/S0740-5472\(02\)00233-7](https://doi.org/10.1016/S0740-5472(02)00233-7); Note that we did discuss on early WG call that an individual level of burnout would likely be more appropriate.

O9. People who work here are determined to implement [intervention].	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O10. People who work here feel confident that they can coordinate tasks so that implementation goes smoothly.	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O11. People who work here are motivated to implement [intervention].	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O12. People who work here feel confident that they can manage the politics of implementing [intervention].	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
Organizational Climate: Stress (Organizational measure)	
TCU Survey of Organizational Functioning/Stress (JCOIN 1.0 optional measure) ⁵	
O13. Staff members are under too much pressure to do their jobs effectively.	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O14. Staff members often show signs of stress and strain.	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O15. The heavy workload here reduces agency effectiveness.	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree
O16. Staff frustration is common here.	1 = Disagree 2 = Somewhat disagree 3 = Neither agree nor disagree 4 = Somewhat agree 5 = Agree

⁵ Source: TCU Survey of Organizational Functioning

7 IMPLEMENTATION OUTCOMES (ORGANIZATIONAL MEASURE)

ACCEPTABILITY, APPROPRIATENESS, & FEASIBILITY (JCOIN 1.0 OPTIONAL MEASURE) ⁶	
OPTIONAL	
Instructions: When using this measure, substitute a short-hand name for the intervention/program into each item in place of [intervention]. If needed, define the intervention at the beginning of the section.	
Appropriateness	
11. [Intervention] seems fitting.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
12. [Intervention] seems suitable.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
13. [Intervention] seems applicable.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
14. [Intervention] seems like a good match.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
Feasibility	
15. [Intervention] seems implementable.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree

⁶ Weiner, B. J., Lewis, C. C., Stanick, C., Powell, B. J., Dorsey, C. N., Clary, A. S., Boynton, M. H., & Halko, H. (2017). Psychometric assessment of three newly developed implementation outcome measures. *Implementation Science*, 12, 108. <https://doi.org/10.1186/s13012-017-0635-3>; Note: 9 hubs are using at least one of these questions.

I6. [Intervention] seems possible.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
I7. [Intervention] seems doable.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
I8. [Intervention] seems easy to use.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
Acceptability	
I9. [Intervention] meets my approval.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
I10. [Intervention] is appealing to me.	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
I11. I like [Intervention].	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree
I12. I welcome [Intervention].	1 = Completely disagree 2 = Disagree 3 = Neither agree nor disagree 4 = Agree 5 = Completely agree

8 BJA CASCADE OF CARE QUESTIONS

OPIOID TESTING, SCREENING, AND TREATMENT SURVEY QUESTIONS (DERIVED FROM CJ-3, ITEM 24)	
OPTIONAL	
Instructions: <i>The following questions ask about routine practices in any jail confinement facilities operated by your jurisdiction. Include testing, screening, and treatment conducted either on or off facility grounds.</i>	
Screening and Identification	
CC1. During intake, does your jail conduct routine urinalysis testing for the detection of opioids?	☐ Yes ☐ No
CC2. During intake, does your jail screen inmates for opioid use disorder using a questionnaire or interview?	☐ Yes ☐ No
Education and Counseling	
CC3. Does your jail provide overdose education to inmates identified as having opioid use disorder?	☐ Yes ☐ No
CC4. Does your jail initiate behavioral or psychological treatment for inmates identified as having opioid use disorder?	☐ Yes ☐ No
Withdrawal Management	
CC5. Does your jail provide medications for the treatment of opioid withdrawal symptoms (e.g., clonidine, lofexidine, methadone, or buprenorphine)? <i>(Exclude non-prescription or over-the-counter medications.)</i>	☐ Yes ☐ No
Medication for Opioid Use Disorder (MOUD / MAT)	
CC6. Does your jail provide medication-assisted treatment (MAT) for opioid use disorder using FDA-approved medications (e.g., methadone, buprenorphine, naltrexone)?	☐ Yes ☐ No
CC6a. If YES to CC6, Does your jail continue MAT for inmates who are admitted with a current prescription for buprenorphine or naltrexone, or who were receiving methadone prior to admission?	☐ Yes ☐ No
CC6b. If YES to CC6, Does your jail initiate MAT for inmates identified as having opioid use disorder?	☐ Yes ☐ No
Pain Management Continuity	
CC7. Does your jail continue providing prescription opioids to inmates with acute or chronic pain who are admitted with a current prescription from a health care professional?	☐ Yes ☐ No
CC8. Does your jail initiate the provision of prescription opioids to inmates to treat acute or chronic pain?	☐ Yes ☐ No
Overdose Prevention and Reentry Linkage	
CC9. Does your jail provide overdose reversal medication (e.g., naloxone/Narcan) to inmates with opioid use disorder to take with them at the time of release?	☐ Yes ☐ No
CC10. Does your jail link inmates with opioid use disorder to medication-assisted treatment in community care upon release?	☐ Yes ☐ No

9. CORE ORGANIZATIONAL-LEVEL METADATA (to be completed by research staff)

Variable	Label
MD1. Organizational ID/ Site ID (i.e., a brief internal code for the organization or site that is being surveyed)	
MD2. Type of Organization	1. Prison 2. Jail 3. Community supervision (e.g., probation, parole) 4. Courts 5. Pretrial services 6. Law enforcement 7. Crisis response / first responders 8. Child welfare 9. SUD treatment services 10. Other healthcare services 11. State agency 12. Recovery service provider 13. Re-entry service provider (delivers coordinated supports specifically to help people successfully transition from incarceration back into the community) 14. Other: Please describe
MD3. Intercept placement: 0-5	1. Intercept 0: Community Services – Focuses on prevention and diversion, such as mobile crisis teams, co-responders, and community-based treatment, before interaction with law enforcement. 2. Intercept 1: Law Enforcement – Involves police-based diversion, crisis intervention teams (CIT), and specialized training for officers to handle behavioral health crises. 3. Intercept 2: Initial Detention & Court Hearings – Focuses on screenings, assessments, and early diversion opportunities during booking and initial hearings. 4. Intercept 3: Jails & Courts Intercept 0: Community Services – Focuses on – Targets individuals in custody, utilizing mental health courts, drug courts, and jail-based treatment to avoid further incarceration. 5. Intercept 4: Reentry –Planning for people leaving jail or prison, ensuring connection to services, housing, and treatment. 6. Intercept 5: Community Corrections – Specialized supervision (e.g., probation/parole) and support services to prevent recidivism. 7. Other: Please describe
MD4. Grantee No	Grantee grant no. from NIDA (will be recoded in shared data)
MD5. Grantee Subsite No.	If applicable, subsite number within the grant – else use 1 (main site).
MD6. Date of study enrollment	Date of study enrollment (e.g., date of consent) (will not be made available in shared data)
MD7. Year of study enrollment	Year in which participant was enrolled
MD8. Financial quarter in which participant was enrolled	Financial quarter in which participant was enrolled (Jan - Mar = 1; Apr – June = 2; July – Sept = 3; Oct - Dec = 4).

MD9. Month of enrollment	Month in which participant was enrolled.
MD10. State of enrollment	U.S. state abbreviation of the site where participant was initially enrolled.
MD11. Current study status	Current status of study participant: 1. On study 2. Dropped out / Lost to follow up 3. Withdrawn by investigator 4. Left organization 5. Deceased 6. Completed study
MD12. Days on study	Number of days participant spent on study following enrollment (for those still on study, number of days from enrollment to current date).
MD13. Treatment / Intervention arm	Treatment arm to which participant was randomized (hub specific)
MD14. Interview number	Sequential number corresponding to each sequential interview for this individual study subject (e.g., 'Baseline' = 1, 'Three months' = 2, 'Six months' = 3, etc.), regardless of the number of months between.
MD15. Interview wave number	Sequential number corresponding to the intended wave of the study data collection (e.g. 'Baseline = 1', 'Wave 2 = 2', 'Wave 3 = 3', etc.), regardless of the sequential number of the individual respondent interview (e.g. a study subject can have their Baseline interview at 'Wave 3' assessment timepoint).
MD16. Intended interview timepoint (in months)	Intended interview timepoint in months since the study onset (e.g., 'Baseline', '3 month', '6 month', etc.). This corresponds to the study wave and not the sequential number of the individual subject interview (e.g. a study subject can have their Baseline interview at '6 months' / 'Wave 3' assessment timepoint).
MD17. Interview days from baseline	Number of days between the baseline assessment and the current assessment/current date; for baseline interview enter zero.
MD18. Administration method	How were the questions administered? 1. Self-administered 2. Orally administered by staff 3. Orally administered by others Administered some other way (describe)
MD19. Administration mode	What was the mode of administration? 1. pen and paper; 2. computer; 3. telephone; 4. Zoom/Teams; other (describe)
MD20. Location (ZIP code)	Zip code of organization's site/location that employs the staff from which data are being collected: (can be recoded into county or census tract later; n/a if it's a state agency HQ)